

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$37,510,869
Prenatal arm	\$11,818,493
Postnatal arm	\$25,692,376
Administrative load (8%)	\$3,000,869
Appropriation total (benefit + admin)	\$40,511,738
Sampling 90% CI	\$28,149,437 - \$46,872,300
Assumption band	\$22,506,718 - \$47,825,833

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) · 6-mo postnatal	\$4,500	\$37,510,869	\$40,511,738	16,443
Universal · 6-mo postnatal	\$4,500	\$63,874,931	\$68,984,926	28,000
Universal · 12-mo postnatal	\$7,500	\$107,624,884	\$116,234,875	28,000

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$25,692,376
Total cost (12-month design)	\$63,203,245

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

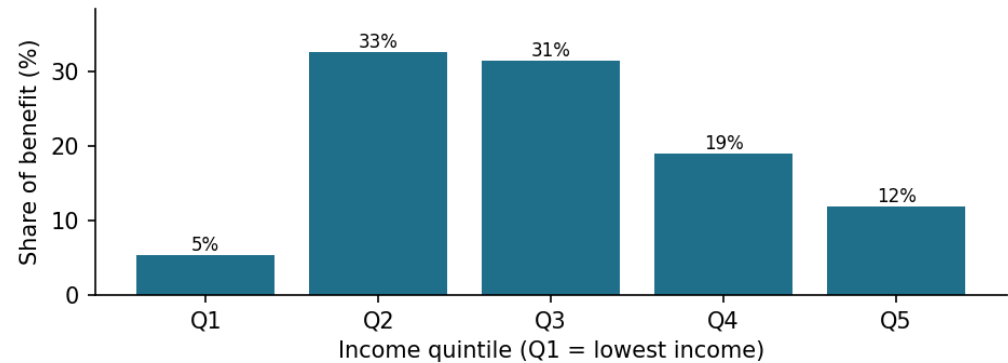
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$15,560,540
Prenatal / Postnatal	\$6,401,684 / \$9,158,856
Postnatal deferred to next FY	\$4,757,847
Year-1 as % of steady state	41%
Ramp sensitivity (6 / 12 / 18 mo)	\$24,343,526 / \$15,560,540 / \$10,373,693

Household reach

Eligible families (weighted)	7,340
Expected recipients / year	16,443
Expected pregnancies / infants	7,879 / 8,564
Avg benefit per recipient	\$2,281

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,182	110,267	865	\$1,973,543	5.3%
Q2	\$57,028	110,263	5,367	\$12,242,728	32.6%
Q3	\$96,722	110,317	5,165	\$11,783,736	31.4%
Q4	\$154,809	110,231	3,107	\$7,086,815	18.9%
Q5	\$334,087	110,343	1,939	\$4,424,047	11.8%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$5,430,664	\$1,711,031	\$3,719,633	2,381
Honolulu	\$27,447,245	\$8,647,762	\$18,799,483	12,032
Kauai	\$2,840,841	\$895,059	\$1,945,781	1,245
Maui	\$1,792,119	\$564,640	\$1,227,479	786

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.